Ovarian tumor

By Dr.Tanvi Vasalla

- Mrs. v 64 yr old woman wife of Mr.G
- Living at vadodra
- Educated upto class 9
- Homemaker
- Class 3 socioeconomic status

- Shes a postmenopausal women with complains of abdominal distension and pain in abdomen for past 2 months . She also gives history of loss of appitite for past 2 months.

Hopi:

- Patient was apparently healthy 2 months back , she started developing abdominal distension that progressively increased over past 2 months .there is history of associated dull aching pain in abdomen with no specific aggravating or relieving factors.
- She gives history of bloating sensation and dyspeptic symptoms for past 2 months.
- There is history of decreased appitite for past 2 months, there is no history of weight loss.
- No h/o postmenopausal bleeding
- No h/o fever or abnormal discharge p/v
- No h/o bowel , bladder disturbances

Menstrual History:

- Attained menarche at 12 yrs
- Had regular cycles 4/30 days menstrual cycle.
- Attained menopause at age of 50 yrs.

Marital and sexual history:

- Got married at age 24 yrs
- Married for past 40 yrs

Obstetric history:

- P1L1 with 1 living child
- Spontaneous conception at age of 29 yrs she had ftnd and post natal period was uneventful
- She did not use any contraception following delivery however did not hav any further pregnancies

Past medical/surgical history:

- No h/o diabetes. She is known hypertensive for past 10 years and is on medications
- No h/o tuberculosis, jaundice, thyroid disorders.
- No h/o surgery or blood transfusion
- No h/o stds
- No h/o drug intake for any chronic illness

Personal history

- No h/o alcohol intake , smoking, tobacco and drug addictions
- No h/o drug allergy
- Sleep normal
- Appetite reduced for past 2 months.

Family history:

- There is no h/o HT,DM,TB in family
- No h/o genital malignancy

Examination:

- General examination:
- Patient is comfortable at rest
- Average built well nourished
- Ht 156 cm wt 68 kg
- Pallor is present
- No icterus, no pedal edema, no lymphadenopathy

vitals

- Afebrile pulse is 86/min bp is 120/80
- Breast and thgyroid normal
- Spine is normal

Abdominal examination:

- Inspection:
- Abdomen appears distended skin over abdomen is stretched and shiny
- Umblicus everted midline in position
- No scar , sinuses oe engorged veins seen
- No cough impulse at hernial orifices

- Palpation:
- Mass arising from pelvis enlarged upto 24 week size with variegated consistency . Irregular surface , restricted mobility
- Non tender on palpation
- Margins of mass are not well defined , lower border of mass is not palpable
- No hepatosplenomegaly
- No fluid thrill present

Percussion

- Dull note over mass . Shifting dullness could not be elicited.

Pelvic examination:

- External genitilia:healthy
- PER Speculumexamination- caervix and vaginal healthy
- Bimanual examination:
- Mass arising from pelvis 24 week size firm in consistency irregular surface
- Mass is tripped through right fornix
- Mass is felt separately from uterus and uterus is normal in size.
- Movement of mass not transmitted to cervix

Rectal and rectovaginal examination:

- Nodules are felt in pouch of douglas
- Rectal muscosa and parametrium are free

summary

- 64 yr postmenopausal woman P1l1 with complain of progressively increasing abdominal distensionwith diffuse pain abdomen and dyspeptic symptoms for 2 months.associated loss of appitite is present.
- On examination 24 week sized ill defined non tender mass with variegated consistency , irregular surface and restricted mobility is felt arising from pelvis.
- Bimanual examination confirms inspection finding and mass is tipped from rt fornix, mass is felt separately from uterus and movement of mass is not transmitted to cervix. On rectal and rectovaginal examination . Pod nodules are felt.

Case diagnosis

- 64 yrs para 1 live 1 post menopausal woman with abdominopelvic mass , considering he history of rapidly progression of symptoms and presentation within a short time span and examination findings it is most likely a case of ovarian tumor probably malignant.